

APO-LORATADINE
Loratadine Tablets 10mg
Histamine H₁ Receptor Antagonist

CLINICAL PHARMACOLOGY

Loratadine is a long-acting tricyclic antihistamine with selective peripheral H₁ receptor antagonistic activity. It exhibits a dose-related inhibition of the histamine-induced skin wheal and flare response in humans which is rapid in onset, is apparent at two hours and persists throughout the 24 hour observation period. Single oral dose up to 160 mg and repeat daily doses of 40 mg for up to 13 weeks were well tolerated with the incidence of sedation and dry mouth being no different from placebo.

INDICATION AND CLINICAL USE

APO-LORATADINE (loratadine) is indicated for the relief of symptoms associated with seasonal and perennial allergic rhinitis, such as sneezing, nasal discharge and itching, and ocular itching and burning, and for the relief of symptoms and signs of chronic urticaria and other allergic dermatologic disorders.

CONTRAINDICATIONS

APO-LORATADINE (loratadine) is contraindicated in patients who have shown hypersensitivity or idiosyncrasy to the drug or its components.

WARNINGS / PRECAUTIONS

Use in Pregnancy and in Nursing Mothers

The safe use of loratadine during pregnancy or lactation has not been established and therefore the compound should be used only if the potential benefit justifies the potential risk to fetus or infant.

Use in Children

The safety and efficacy of loratadine in children younger than 2 years of age have not been established. Long term safety and efficacy of loratadine in children between the ages of 2 and 12 have not been demonstrated. Therefore, it is desirable that APO-LORATADINE (loratadine) not be administered to children between the ages of 2 and 12 for longer than 14 days, unless recommended by a physician.

Use in Patients with Liver impairment

Patients with severe liver impairment should be administered a lower initial dose because they may have reduced clearance of loratadine; an initial dose of 5 mg once daily or 10 mg every other day is recommended.

DRUG INTERACTIONS

When administered concomitantly with alcohol, loratadine has no potentiating effects as measured by psychomotor performance studies. Increases in plasma concentrations of loratadine have been reported after concomitant use with ketoconazole, erythromycin or cimetidine in controlled clinical trials, but without clinically significant changes (including electrocardiographic). Other drugs known to inhibit hepatic metabolism should be coadministered with caution until definitive interaction studies can be completed.

Drug/Laboratory Test Interactions: Loratadine should be discontinued approximately 48 hours prior to skin testing procedures since antihistamines may prevent or diminish otherwise positive reactions to dermal reactivity indicators.

ADVERSE REACTIONS

Fatigue, headache, dry mouth, sedation, gastrointestinal disorder, nervousness and hyperkinesia. Rarely, alopecia, anaphylaxis and abnormal hepatic function.

SYMPTOMS AND TREATMENT OF OVERDOSAGE

Somnolence, tachycardia and headache have been reported with overdoses of the conventional loratadine formulation. A single acute ingestion of 160 mg produced no adverse effects. In the event of overdosage, treatment, which should be started immediately, is symptomatic and supportive. Discontinuation of use, gastric lavage or induction of emesis (except in patients with impaired consciousness) and support of vital functions are advised.

The patient should be induced to vomit, even if emesis has occurred spontaneously. Pharmacologically-induced vomiting by administration of ipecac syrup is a preferred method. However, vomiting should not be induced in patients with impaired consciousness. The action of ipecac is facilitated by physical activity and by the administration of 240 to 360 mL of water. If emesis does not occur within 15 minutes, the dose of ipecac should be repeated. Precautions against aspiration should be taken, especially in children. Following emesis, adsorption of any drug remaining in the stomach may be attempted by the administration of activated charcoal as a slurry with water. If vomiting is unsuccessful, or contraindicated, gastric lavage should be performed. Physiologic saline solution is the lavage solution of choice, particularly in children. In adults, tap water can be used; however, as much as possible of the amount administered should be removed before the next instillation. Saline cathartics draw water into the bowel by osmosis and therefore may be valuable for their action in rapid dilution of bowel content. Loratadine is not removed by hemodialysis; it is not known if loratadine is removed by peritoneal dialysis.

DOSAGE AND ADMINISTRATION

Adults and Children 12 years of age and over: One loratadine tablet (10 mg), once daily.

AVAILABILITY OF DOSAGE FORMS

White, oval, biconvex tablet. Scored & engraved LO over 10 on one side, APO on the other.. Available in PVC blisters of 3 x 10's and 100 x 10's.

STORAGE CONDITIONS: Store below 30°C. Protect from light.

Manufacturer: Apotex Inc., 150 Signet Drive, Toronto Ontario, Canada M9L 1T9.

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